



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : كريمه السيد عبده السيد
المحافظة : الجيزه
الحى / المدينة : امبابه
رقم البطاقة : 26010232800121 رقم التليفون : 01149472026
العمر (تاريخ الميلاد) : 23-10-1960 المهنة : ربه منزل
العنوان : قصر النوبه
تاريخ تسجيل الإستماره : 27-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: كريمة السيد عيده السيد
National ID: 26010232800121



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

بعضاً من تحقيق الشخصية

三

• Small size

العصر الذهبي - الجيزة

77.1.77 28.11.77

LK6075850

Y. T. Ojima

77-5-TPA-171

رسالة مختارة

五

Value

5

البطاقة سارية حتى ٢٠٣٧/١٠/١٢

الى من يهمه الامر / مستشفى / امجاله العليه بالاس

نصية طبية وبعد،،،،

بالاشارة الى قانون الهيئة العامة للتأمين الصحي الشامل رقم 2 لسنة 2018 ، والذي يتضمن بوجوب خضوع المقيمين بمحافظات التطبيق اجباريا بالمنظومة وفقا لمحل الاقامة ببطاقة الرقم القومي داخل احدي محافظات التطبيق، وبلاشارة الى بدء التشغيل الفعلي بتاريخ 2025/1/1
لمنظومة التأمين الصحي الشامل بمحافظات التطبيق

برجاء التفضل بالعلم بأنه قد تم الاستعلام عن بيانات المواطن / كرم السيد السيد / تحمل بطاقة الرقم المرفقه طيه والتي قد تم اصدارها في تاريخ بمح / 2025/1/1
الاقامة في محافظة وبناء علي ما سبق في ان المستفيد المذكور من غير الخاضعين لمنظومة التأمين الصحي الشامل.

وتفضلوا بقبول فائق التقدير والاحترام،،،

مدير فرع

مدير ادارة

مدير ادارة

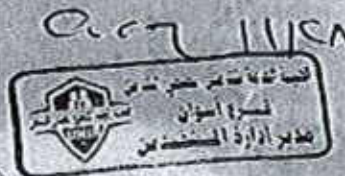
المستفيدين

التسجيل والاصدار

م. ط. أبو الجوز

م. ط. أبو الجوز

م. ط. أبو الجوز





Patient's Name: Karima Elsayed Abdo Elsayed
Date: 2026-05-02
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known case of breast CA, compared to the previous study dated 3-1-2026, the current study showed:

1. Chest:

- No appreciable time interval changes regarding the previously seen right breast spiculated mass measuring 4 x 2.4 cm
- No pulmonary masses, sizable nodules, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous low CT texture. Shows two hepatic focal lesions at segment IV/V largest measuring 27x24mm and smallest 16 x11mm both showing portovenous enhancement and progressive persistent filling in the delayed phase likely in keeping with liver hemangiomata.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Still noted bulky right adrenal gland.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and left suprarenal gland.
- Still noted anterior abdominal wall midline thick walled collection measuring 8.7 x 4.7 x 6.3 cm.
- Still noted left labial lesion is measuring 3.7 x 2.8 cm
- Still noted diffuse sclerotic lesions of axial and appendicular skeleton
- Degenerative changes of the dorso-lumbar spine.

OPINION:

- Stationary course as regard the hepatic focal lesions with possibility of being liver hemangiomata for correlation with dynamic MRI liver study.
- Stationary course regarding the rest of the study findings.

Dr Abubakr Hejazi MSc FEBR
Dr.sherif Wagdy

Name: Karima El-Sayed Abdul El-Sayed
Branch: Dokki
Ref. Physician Prof: Goustav Roussey Egypt

Date: 21/04/2026 07:18
Medical No.: 355663
Inv. Company: شركة مستشفيات

Tc^{99m} MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

66 year-old female patient, with history of metastatic right breast cancer to the bone received chemotherapy, complains of bony pain and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

Compared to the previous bone scan dated 10/09/2025, the current study revealed marked regression of the metastatic bone disease evidenced by:

- Marked regression in term of size and tracer uptake intensity is noted at the previous lesions involving the skull, sternum, both shoulders, both humeri, clavicles, dorso-lumbar vertebrae, both scapulae, bony pelvis and both femora.
- No newly developed lesions could be elicited on the current study.
- Osteoarthropathic changes are noticed around both knees.

OPINION

- Remarkable regression of the metastatic bone disease with no newly developed ones as described above indicating good response to the given therapy.
- Follow up bone scan would be advisable.

Much Obligated
Dr. Alaa Nagy, MD
Nuclear medicine

Name : KAREEMA AL SAYED +C

Date : 03-01-2026 01:31

Branch : Portable

Medical No. : GRI-26010232800121

Ref.Physician Prof :

Ins.Company :

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
& PELVIS REVEALED: -**

as compared to last study dated 27/9/2025

1. Chest:

- Right breast spiculated mass connected to skin measuring 4 x 2.4 cm
- No pulmonary masses, sizable nodules, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- Still noted anterior abdominal wall midline thick walled collection measuring 8.7 x 4.7 x 6.3 cm with no appreciable changes with subcutaneous calcified lesion .
- Left labial mass lesion is still noted measuring 3.7 x 2.8 cm
- The liver is enlarged with homogenous low CT texture. An ill defined lesion measuring 15 x 10 mm is seen at segment IV b /V with no gross contrast enhancement .
- Segment VIII area of fatty infiltration not barely appreciated (no precontrast series available)
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Degenerative changes of the dorso-lumbar spine.
- **Diffuse sclerotic lesions of axial and appendicular skeleton**

OPINION:

Right breast spiculated mass

As compared to last study , the current study revealed stationary course .

Dr. Reham Khedr M.D.

Patient Name : الأمينة / كريمة السيد محمد السيد
Patient No. : 26010232800121
Path No. : 2380/22
Sex : Female
Age : 61 Y
Referring Doc. :

Request No. : 13006026
Request Date : 18/08/2022
Verification Date : 28/08/2022
Financial Category : برنامج دار السلام - مبادرو
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Right breast mass.

Nature of the specimen:

Tru-Cut biopsy.

Gross Examination:

3 greyish white and fatty tissue cores, up to 2cm long. Totally submitted.

• Immunohistochemical technique: Sections prepared from paraffin block for immune- staining using Ber- Mark ULTRA system for ER, PR, HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1).
- Angioinvasion: No.
- Ductal carcinoma in situ: No.
- Additional pathologic findings: No.

Diagnosis:

Right breast mass; Tru-Cut biopsy:

-Invasive duct carcinoma GII.

-IPT (Positive tissue control showed appropriate reactivity):

-ER: Strong nuclear reaction in more than 70% of tumour cells. Positive, score (8/8).

-PR: Weak nuclear reaction in around 20% of tumour cells. Positive, score (4/8).

-Her-2: Weak incomplete membrane reaction in 10% of tumour cells. Negative, score 1+

-Ki67: less than 1%

Prof. Dr. Lubna Omar

Dr. Abdulrahman Ibrahim

Verified by :

abdelrahman ibrahim

Printing Date : 29/08/2022 12:07:20

Page 1 of 2

Printed By : marwa mohamed yousse

Patient Name: Karima Al Sayed Abdou Al Sayed	Date : 27/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 65 Year(s)
Medical ID: 3396/2022	Pat. ID: 18221

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

(I) Chest:

- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Enlarged thyroid with heterogeneous texture ,for sonographic assessment.

(II) Abdomen and pelvis:

- Midline anterior abdominal wall intramuscular thick-walled collection is noted , measuring 9.2x3.9x6.4 cm along its maximum TS,AP and CC dimensions , with subcutaneous partially calcified lesions , likely focal fat necrosis.
- Left labial isodense lesion is noted , measuring 3.9x2.9 cm ,for further assessment by post contrast MRI pelvis .
- The liver is enlarged in size showing homogenous CT density with right hepatic segment 5 and left hepatic segment 4b small focal lesions , measuring 11x8 mm in diameter each with nodular arterial enhancement and complete enhancement in the delayed phase.
- Right hepatic segment 8 areas of focal fatty infiltration.
- Cholecystectomy clips are noted.
- Patent and homogeneously enhancing portal vein.

- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, apart from small renal cortical cysts, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- Diffuse heterogeneous osseous texture is noted ,for bone scan correlation.

OPINION:

Status postoperative showing the above detailed changes.

- Hepatomegaly with two right hepatic benign-looking focal lesions , likely small hemangiomas.
- No CT evidence of pulmonary depots.
- Diffuse osseous texture alteration.

Dr.Sara Mahmoud, MD

Name : Karima El-Sayed Abdu El-Sayed

Date : 10-09-2025 03:34

Branch : Dokki

Medical No. : 199267

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 64-year-old lady, with history of metastatic breast cancer.

Technique:

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

Considering the prior bone scan dated 09/03/2025:

- Still seen widespread foci of increased tracer uptake, distributed at skull, sternum, shoulder girdles, scapulae, clavicles, humeri, bilateral ribs, spine, bony pelvis and both femora.
- The scan features speaks of superscan pattern, deduced from diminished soft tissue activity and faint visualization of both kidneys.

Impression:

- Metastatic superscan pattern with stationary widespread osseous deposits.

Much oblige
Dr. Alaa Nagi, MD
Nuclear Medicine

24/11/2024 12:35

Patient ID: Name: ALMDT form number: Date: 24/11/2024

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:

1- Name: كريمة السيد جبر العتيق

Sex:

2- Medical number:

Female

33090613

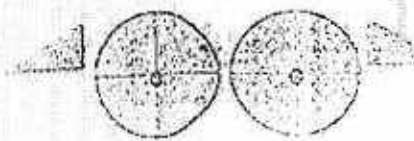
Old Med I

Date:

1396-2022

3-ID number

Patient Age: 64 Y



Family history

Family history

Phone

Fax

01122442985

GMI

DHI

REASON OF MOT

REAL TIME

Marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MOT

24/11/2024

Co-morbidity

Co-morbidity

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging:

Mets Workup:

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	3/3	4/3			
ACKNOWLEDGMENT	ACKNOWLEDGMENT	ACKNOWLEDGMENT	ACKNOWLEDGMENT	ACKNOWLEDGMENT	

T N M
1 bone

Sonomammography

PATIENT HISTORY

9/2022 MBC to bones

femara + zometa + RTH till 7/2024

7/2024 bone scan: DP

shifted to faslodex+ibrance --> now 11/2024 query pancreatic mass > MOH refused

IN VIEW OF +VE FH (her father is prostate cancer) & new query pancreatic lesion
most probably 2nd 1ry pancreatic carcinoma for possibility of BRCA ..EUS & biopsy from pancreatic lesion: hypocellular hgc material with few
a typical large epithelioid cells (suspicious for adenocarcinoma of pancreatic origin)

MDT OUTCOME & PLAN

for PA MRI, labs & TM then surgical MDT

meanwhile to continue on faslodex / ibrance (as there is stationary course)

SIGNATURE

MEDICAL ONCOLOGY

RADIOLOGY

SURGERY

PATHOLOGY

PSYCHIATRY

NUTRITION

Medical No.: 8831

Patient's Name: Karima El-Sayed Abdu El-Sayed

Date: 2026-09-12

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- Retrosternal extension of the thyroid lobes(more on the left side) with focus of macrocalcifications noted inside).
- A spiculated enhancing neoplastic lesion seen in the upper inner quadrant of the right breast,measuring about 27x19mm in dimensions(versus 40x24mm in the previous study). The overlying skin is thickened and reaches maximally to about 7.2mm.
- Normal left breast. Benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 21mm.
- Tiny well-defined subpleural nodules are seen in the anterior segment of right upper pulmonary lobe and the largest one measures about 5.2mm and two other similar nodules are also seen in the anterior segment of both lower pulmonary lobes and the largest one measures about 5.5mm in the right side.
- Linear atelectatic bands are seen scattering in both lung fields.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- Minimal bilateral pleural thickening is seen.
- No pleural or pericardial effusion.
- Average size of the heart with atherosclerotic changes of its big vessels.
- Persistence of the wide spread osteosclerotic bony lesions scattering in the skeleton.
- Average sized liver,measuring about 15 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- The previously-noted hypovascular focal lesion in segment 1Vb and V and each measures about 21x13mm and 26x22 mm in dimensions respectively(versus 27x24 mm and 16x11mm in dimensions respectively).
- No other newly-developed sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 9.7 cm. No focal lesions seen inside.

8831

Name: Karima El-Sayed Abdu El-Sayed

2026-09-12

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



- Enlarged right adrenal gland is still seen and measures about 24x16mm and prominence of the left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.
- Persistence of the right anterior wall collection is still seen and measures about 70x38mm in dimensions (versus 63x47mm in the previous study), with linear partially-calcified lesion also seen in the right subcutaneous tissue ,overlying the right anterior abdominal wall musculatures and measures about 52x14mm in dimensions.

- The left labial lesion could not be evaluated ,being out of view of the current study.

IMPRESSIONS:

A status follow up for a known case with right breast cancer and compared with the previous study done at 02/05/2026, the current examination shows:

- Retrosternal extension of the thyroid lobes(more on the left side) with focus of macrocalcifications noted inside. For US and /or US-guided biopsy.
- An enhancing spiculated neoplastic mass lesion in the upper inner quadrant of the right breast,showing reduced size,denoting a partial regressive course.The overlying skin is thickened with newly-developed tiny well-defined subpleural nodules in the anterior segment of right upper pulmonary lobes and two other similar nodules in the anterior segment of both lower pulmonary lobes ,persistence of the wide spread osteosclerotic bony lesions scattering in the skeleton, enlarged right adrenal gland still seen and prominence of the left adrenal gland and two hypovascular hepatic focal lesions,likely to be metastatic.
- Linear atelectatic bands scattering in both lung fields with minimal bilateral pleural thickening .
- Persistence of the right anterior wall collection still seen, with linear partially-calcified lesion seen in the right subcutaneous tissue with no appreciable changes,denoting a stable course ,overlying the right anterior abdominal wall musculatures.

Prof/Hanan El-Ahwal.

Medical No. : 8831

Patient's Name: Karima El-Sayed Abdu El-Sayed

Date: 2026-09-12

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Dr.Hanan Elahwel

Medical No.: 8831

Patient's Name: Karima El-Sayed Abdu El-Sayed

Date: 2026-09-12

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- No other newly-developed sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 9.7 cm. No focal lesions seen inside.

Name: Karima El-Sayed Abdu El-Sayed

2026-09-12

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



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- No ascites.
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Prof/Hanan El-Ahwal.

Medical No. : 8831

Patient's Name: Karima El-Sayed Abdu El-Sayed

Date: 2026-09-12

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Dr.Hanan Elahwel